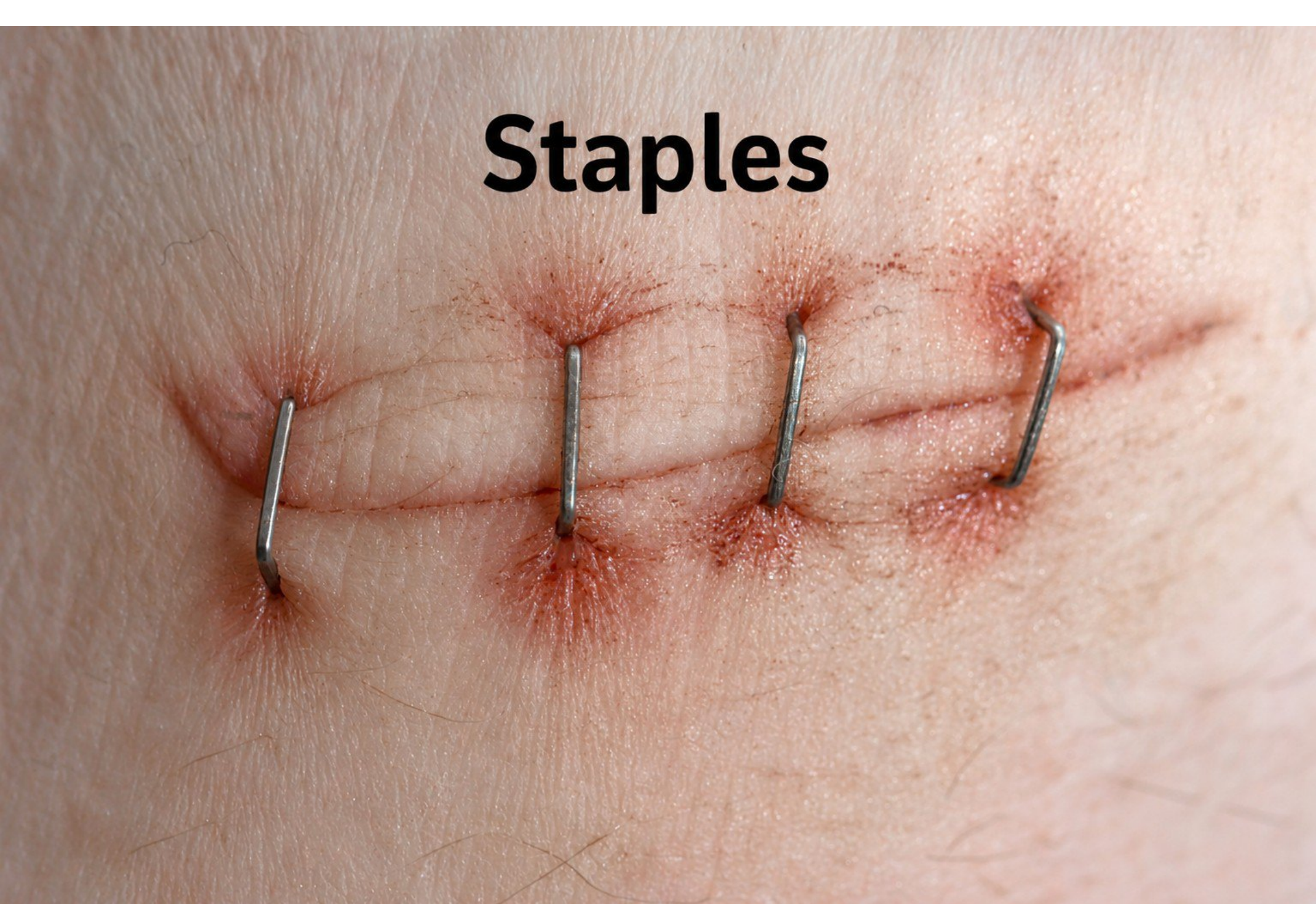




scissors

Staples





Disposable bowl



Extension tubing and stand.



Moisture-proof bag



**Sterile
dressing**



PH testing strips



Chest-physiotherapy

POSTURAL DRAINAGE

- Postural drainage is a technique that uses gravity to help drain mucus from the lungs.
- The patient is placed in different positions so that mucus from different parts of the lungs can drain into the larger airways to be coughed out.



PURPOSES

- Mobilize and loosen secretions
- Drain secretions by gravity
- Prevent or treat pulmonary complications
- Improve ventilation

KEY POINTS

- Use different positions for different areas of the lungs
- Each position is usually maintained for 5–15 minutes
- Follow with chest physiotherapy and encourage coughing
- Monitor the patient for comfort and safety

COMMON POSITIONS

- Semi-Fowler's (Apical)
- Prone (Posterior)
- Supine (Anterior)
- Right and Left Lateral
- Trendelenburg / Reverse Trendelenburg (as needed)

Note: Ensure patient safety, comfort, and monitor for dizziness or breathlessness.

POSTURAL DRAINAGE POSITIONS

(Positions are usually maintained for 5–15 minutes and followed by chest physiotherapy)

1. Apical (Upper Lobes)



Semi-Fowler's position
(Head of bed elevated 45°)

2. Posterior (Upper Lobes)



Prone position
(Pillows under chest and abdomen)

3. Anterior (Upper Lobes)



Supine position
(Head of bed down 15–30°)

4. Lateral (Right Middle Lobe)



Left side-lying position

5. Lateral (Left Lingula)



Right side-lying position

6. Right Lower Lobe (Anterior Basal)



Prone position
(Pillows under chest and abdomen)

7. Left Lower Lobe (Anterior Basal)



Prone position
(Pillows under chest and abdomen)

8. Right Lower Lobe (Posterior Basal)



Supine position
(Head of bed down 15–30°)

9. Left Lower Lobe (Posterior Basal)

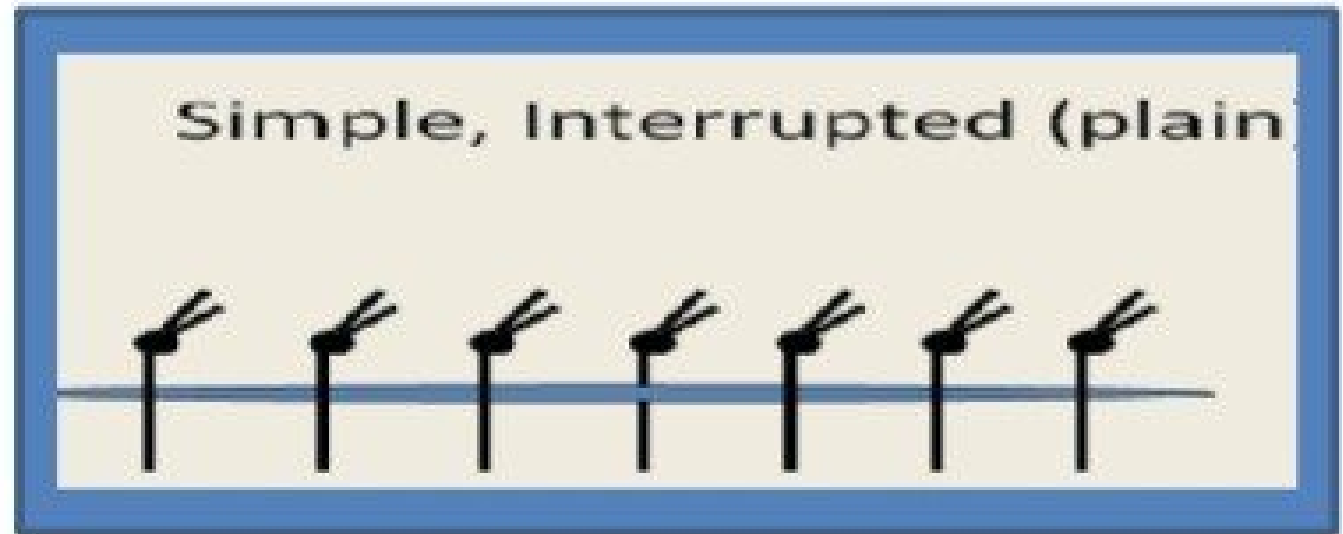


Supine position
(Head of bed down 15–30°)

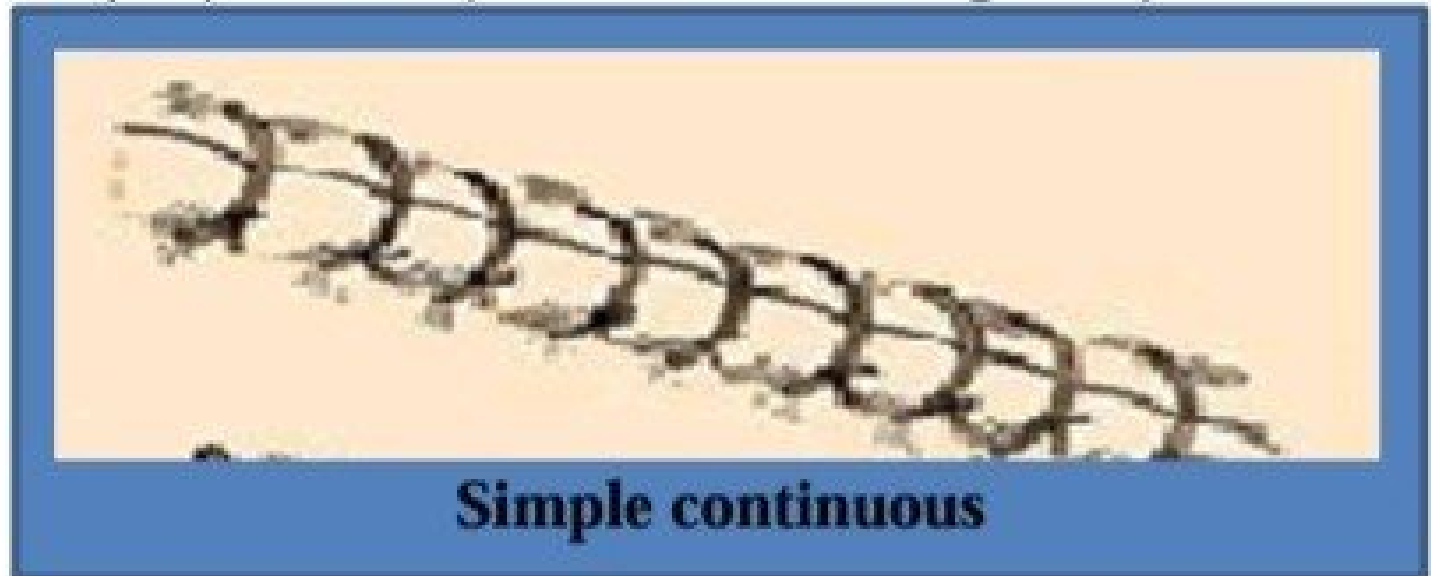
Note: Always ensure patient safety, comfort, and monitor for dizziness or breathlessness.

B- Types of Sutures according to structure techniques:

- 1- Interrupted (plain/ simple as figure1 ,or subcutaneous as figure 2) suture.



- 2- Continuous (plain/ simple ,Mattress, Subcuticular as figure 6) suture.



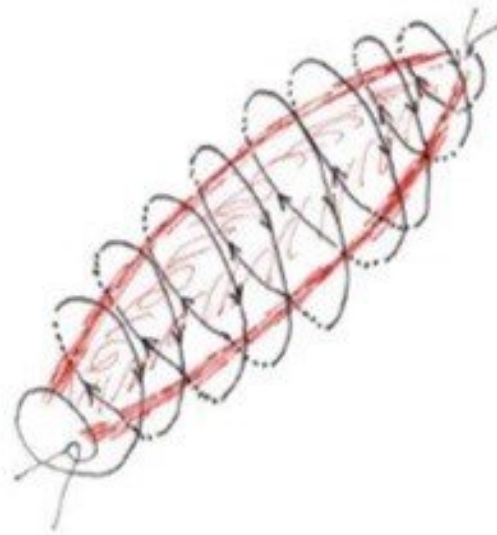


Figure 5. Cutaneous continuous suture

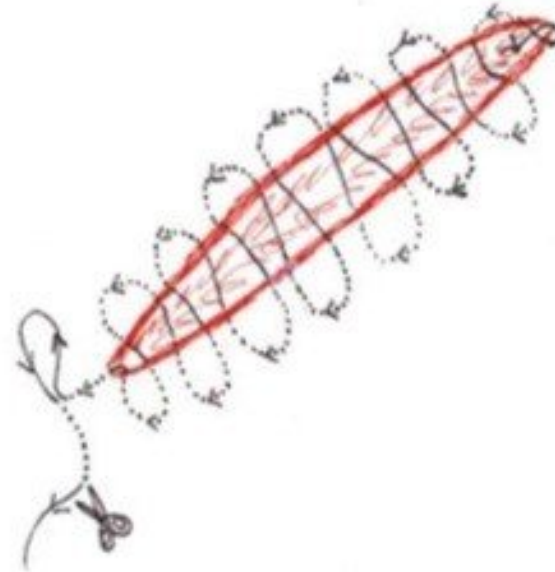
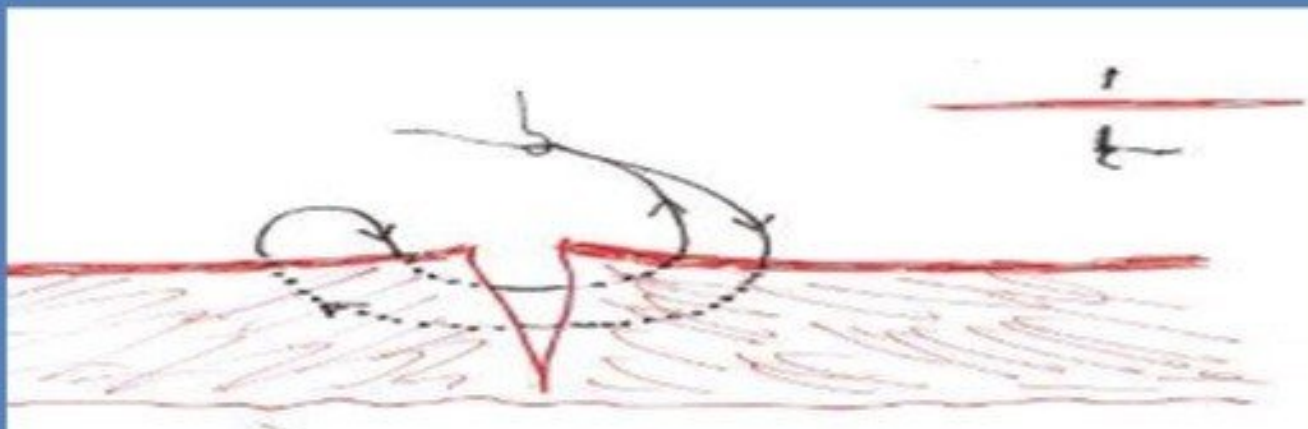


Figure 6. Subcuticular suture

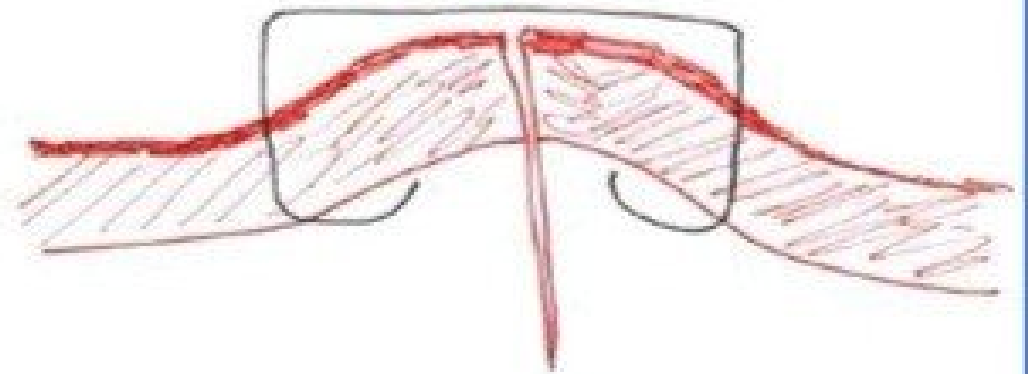
mattress suture: Good for everting wound edges
(neck, forehead creases, concave surfaces)



3. Vertical mattress suture



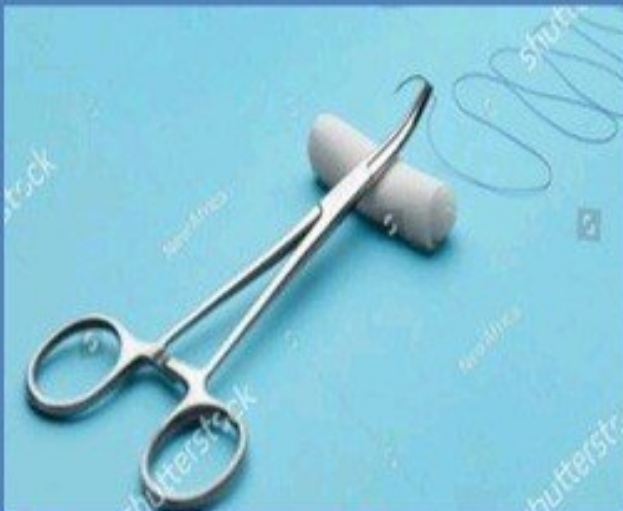
Single-use skin stapler



Wound edge eversion by skin stapler

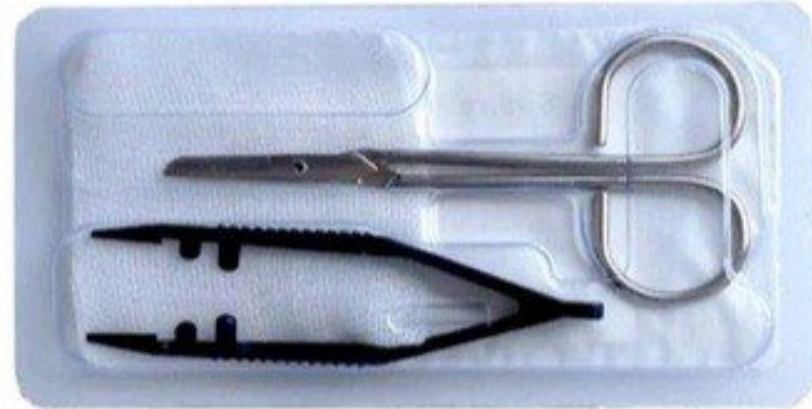
eps and scissors)
ard bag

ze strips
ional)



**Forceps with suture
and thread**

Sterile suture removal set



Clip suture removal

Steri-Strips support wound by distributing tension across wound and eliminate scarring from closure techniques.



- Application of a sterile dressing protects the wound from microorganisms and friction with clothes.

10mL 0.9%
NaCl solution



10mL syringe



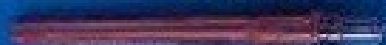
Sterile
gauze



Alcohol swab



Blunt needle



Tourniquet



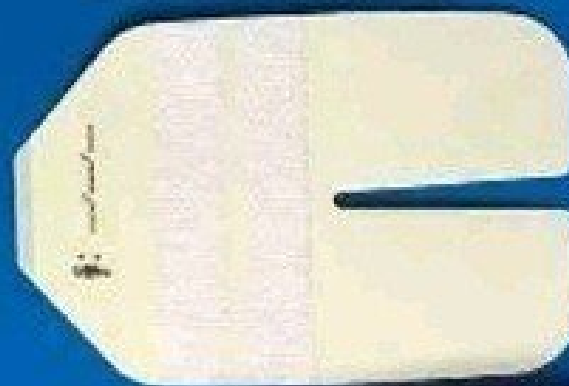
Cannula
(22G)



Extension
set



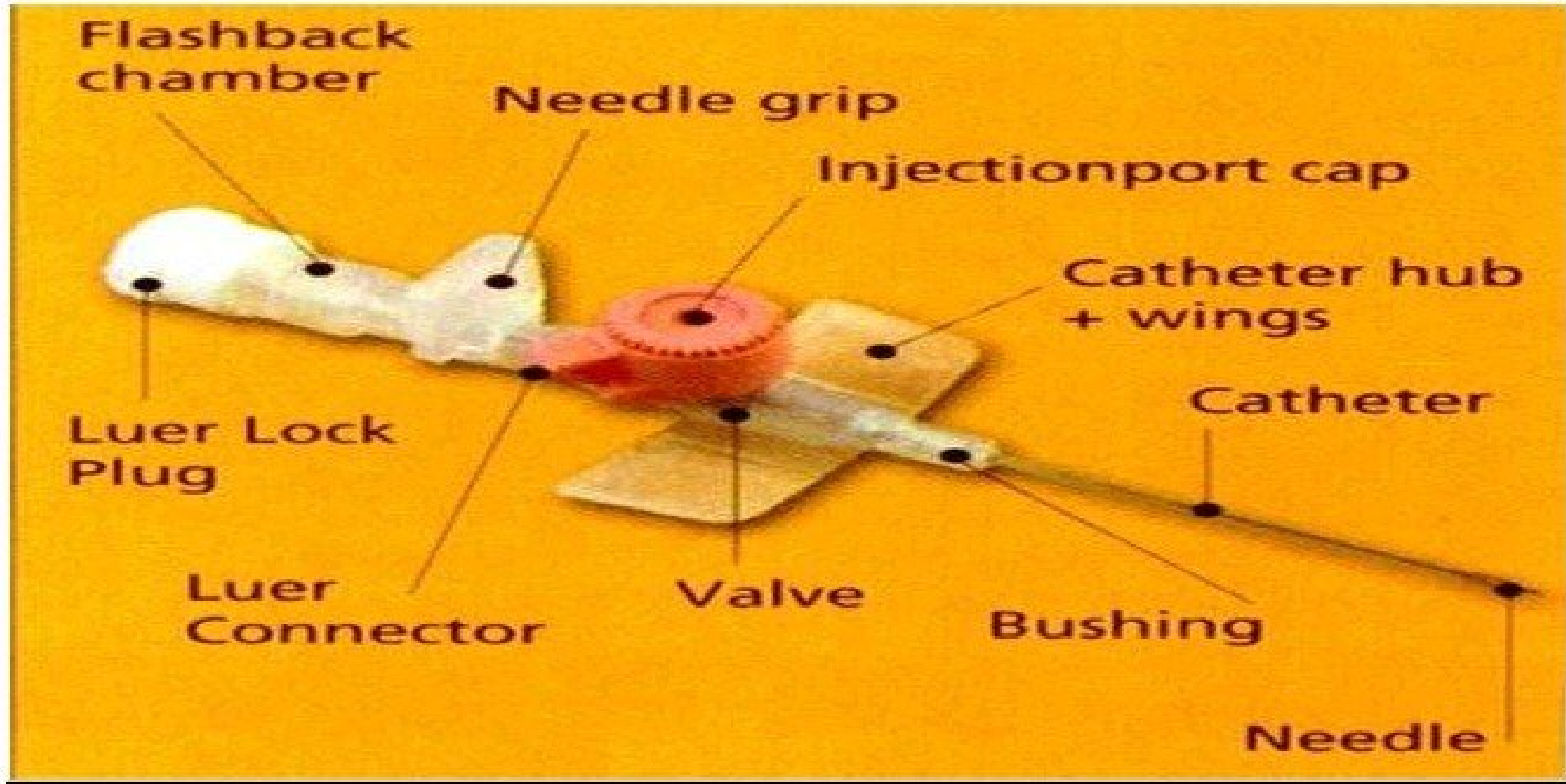
Cannula
dressing



A pair of non-sterile gloves



Parts of cannula:

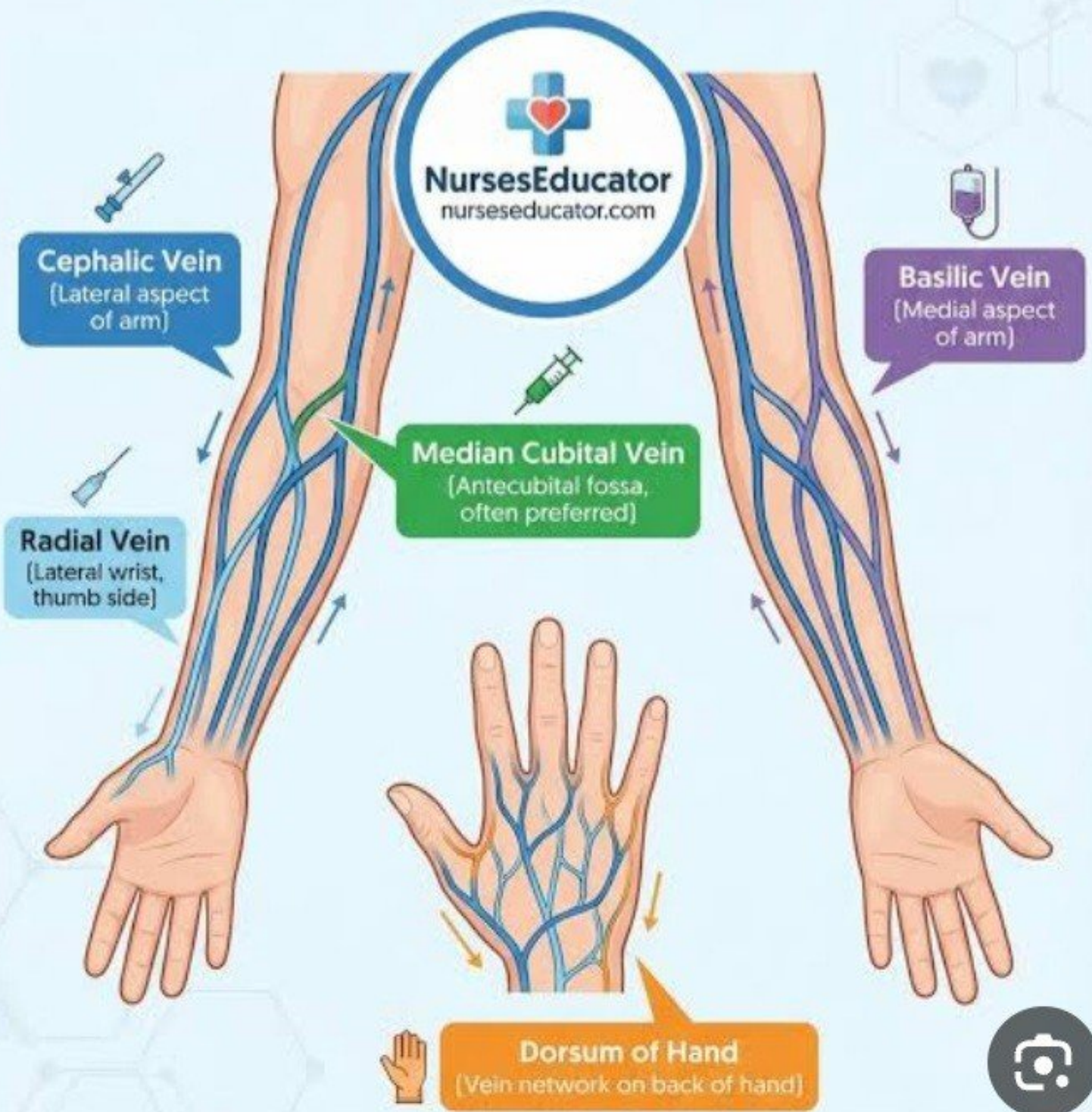




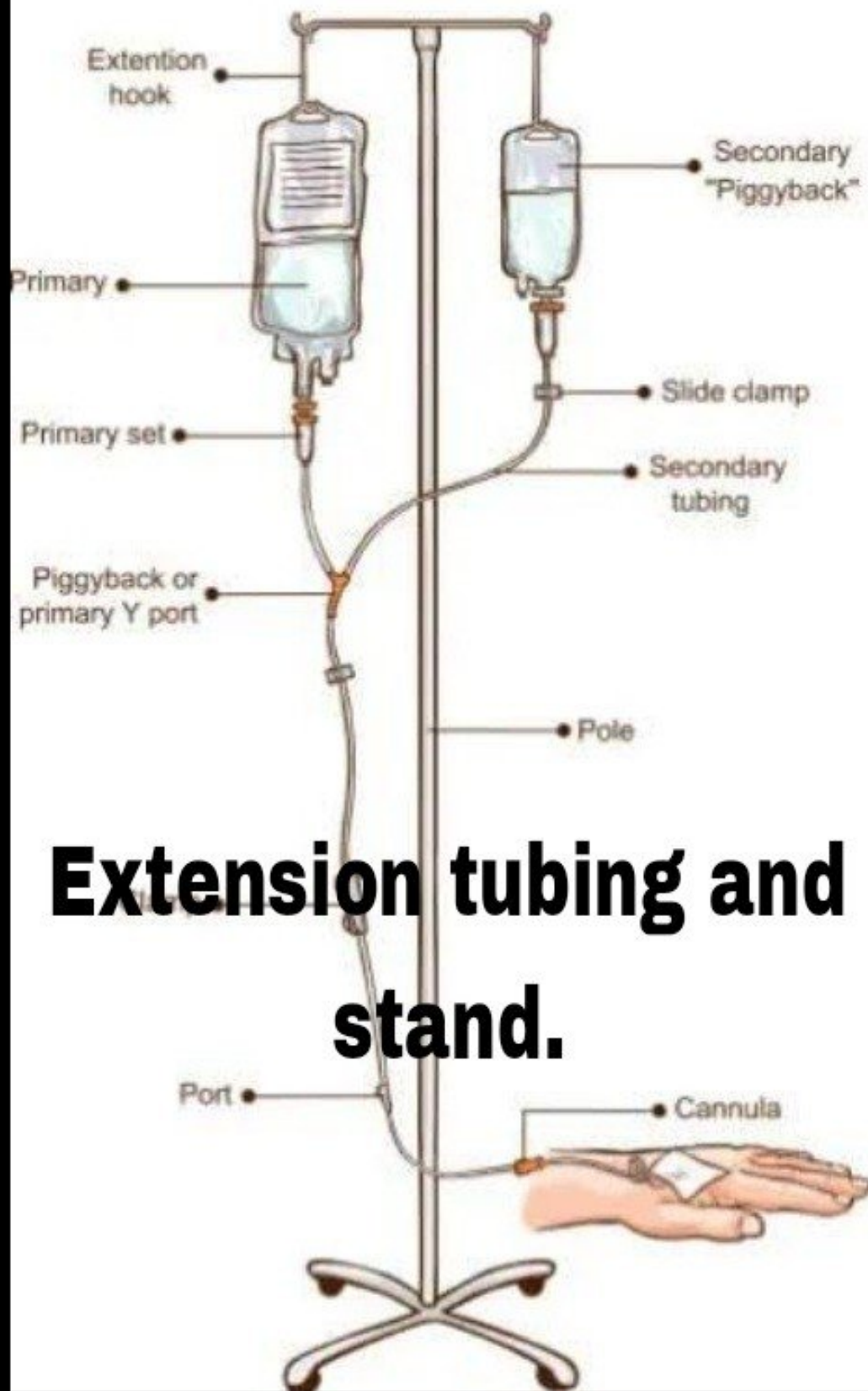
انتقال <

Plastic IV Catheter Needles -
PerfectSeal

Common IV Access Sites: Upper Extremity Veins



IV Secondary Line Infusion



Extension tubing and stand.



انتقال <

How to Use Hospital Bed Iv Line |
TikTok

قد تكون الصور محمية بموجب حقوق النشر. مزيد من المعلومات



< ج

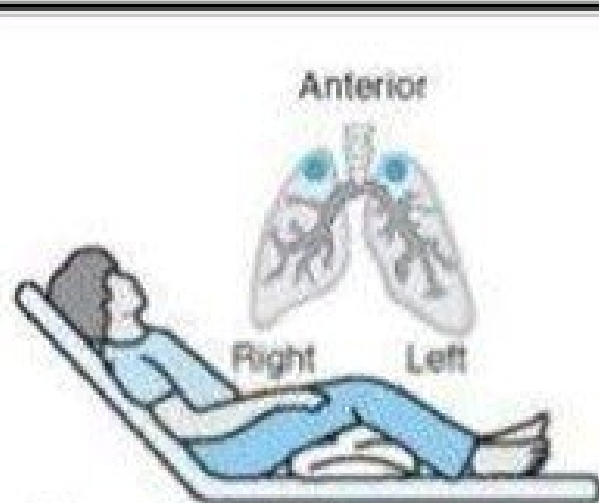
**flush the cannula
with saline**

to a
of a

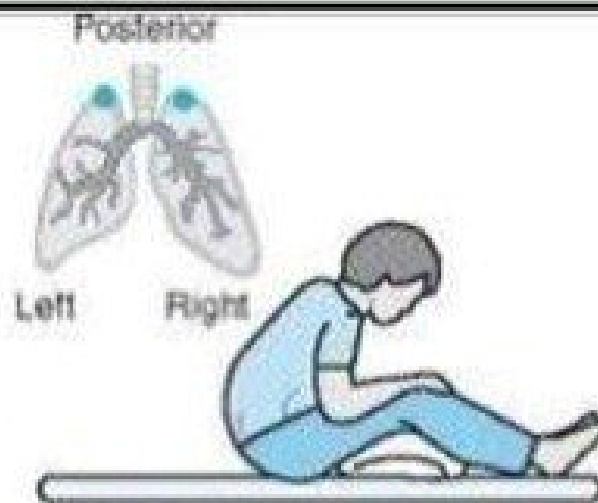
منشئ الصورة:

هل تريد الاطلاع على
قد تكون الصور محمية بموجب حقوق النشر. مزيد من المعلومات

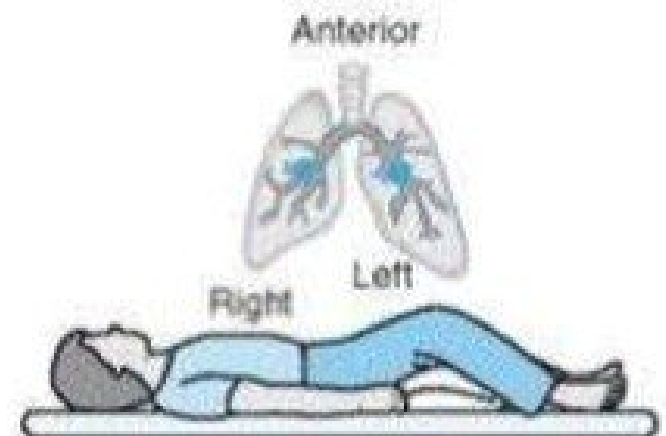
Procedure



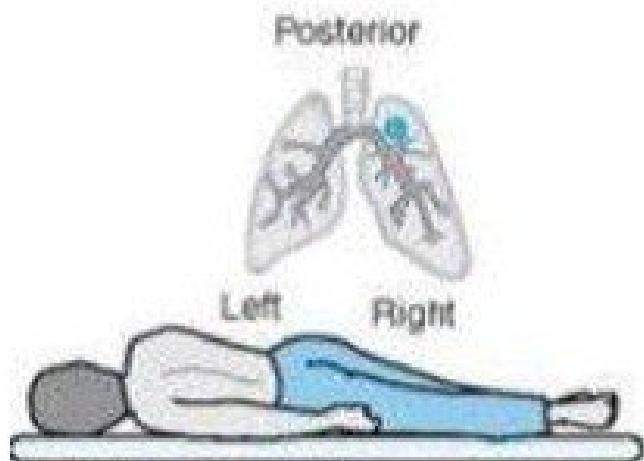
Anterior apical segment
(upper lobes)



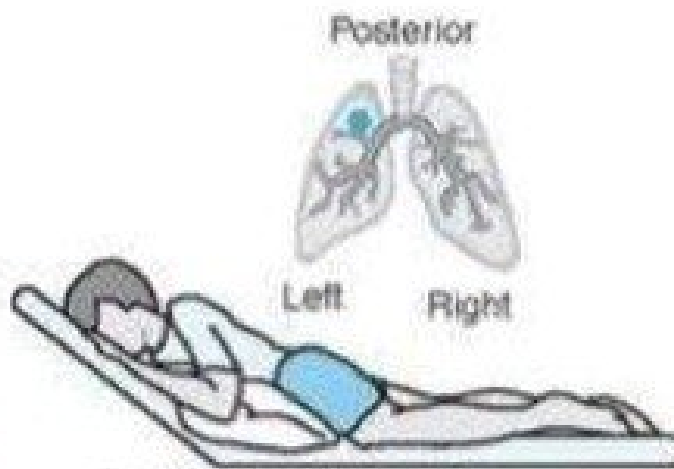
Posterior apical segment



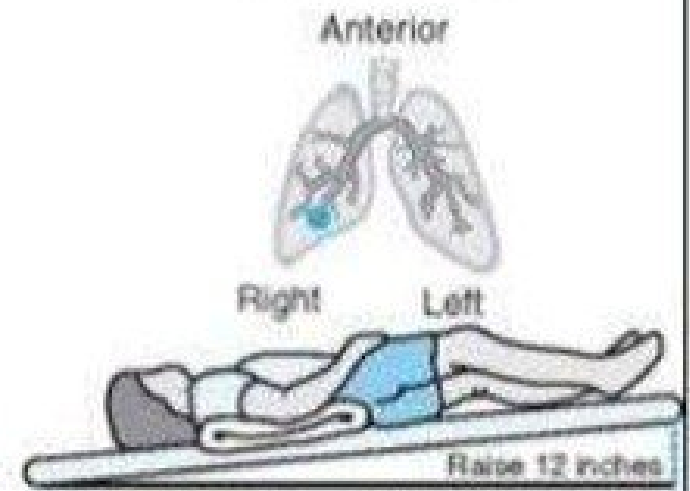
Anterior segments
(upper lobes)



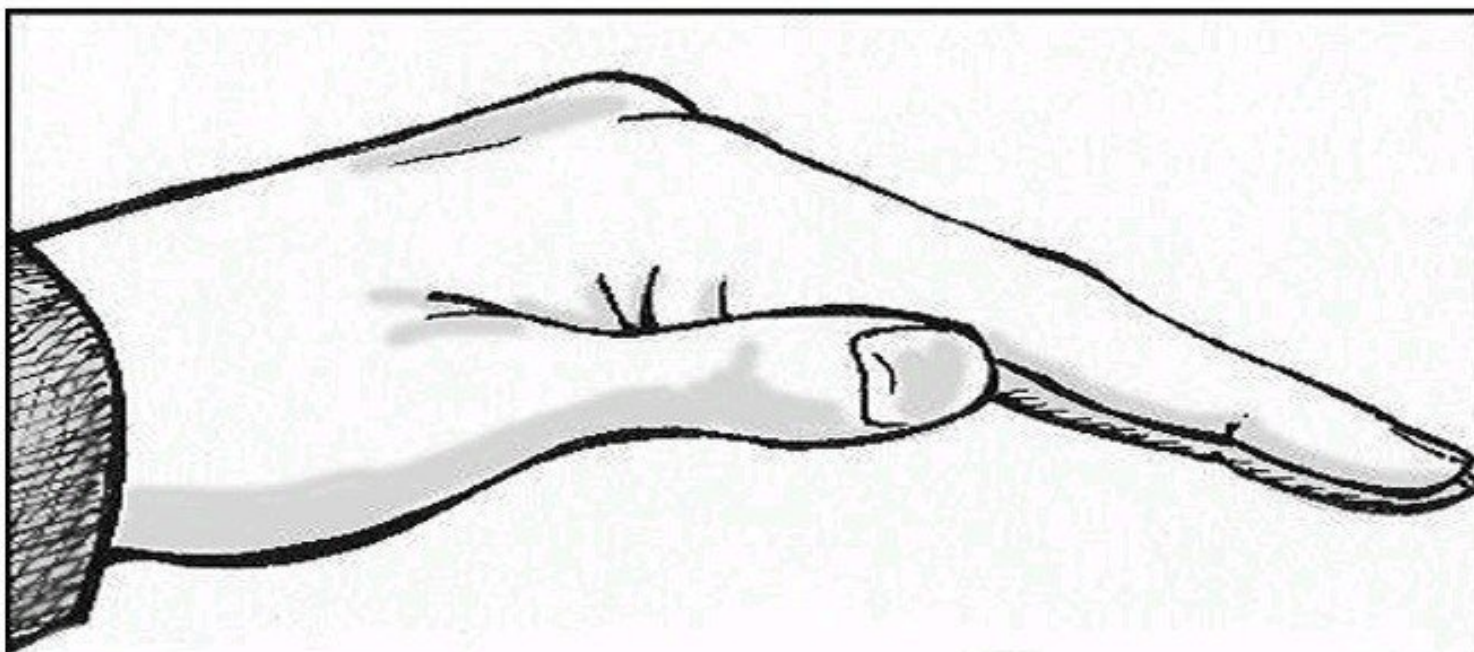
Right posterior segment
(upper lobe)



Left posterior segment
(upper lobe)



Right middle lobe



Correct Hand Position for Chest Percussion

on,
the
hm
lers



Cup-shaped hand

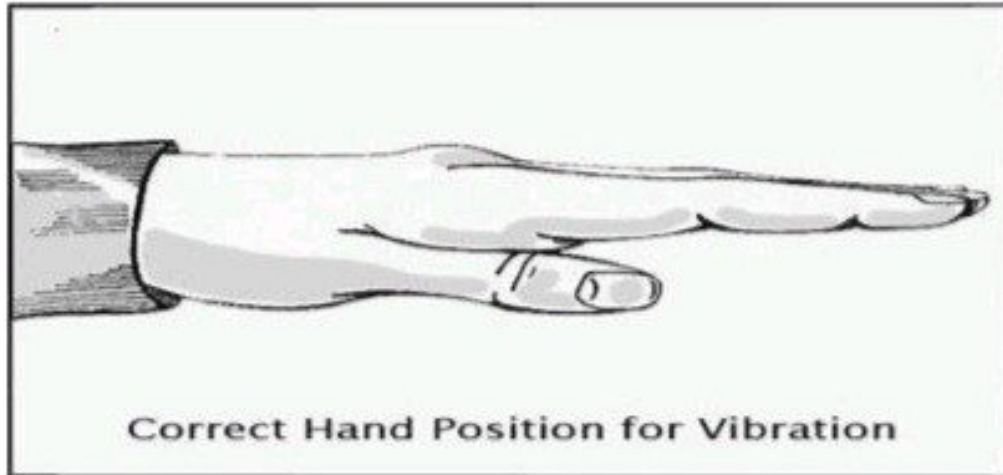


Percussion

did percussing over the spinal

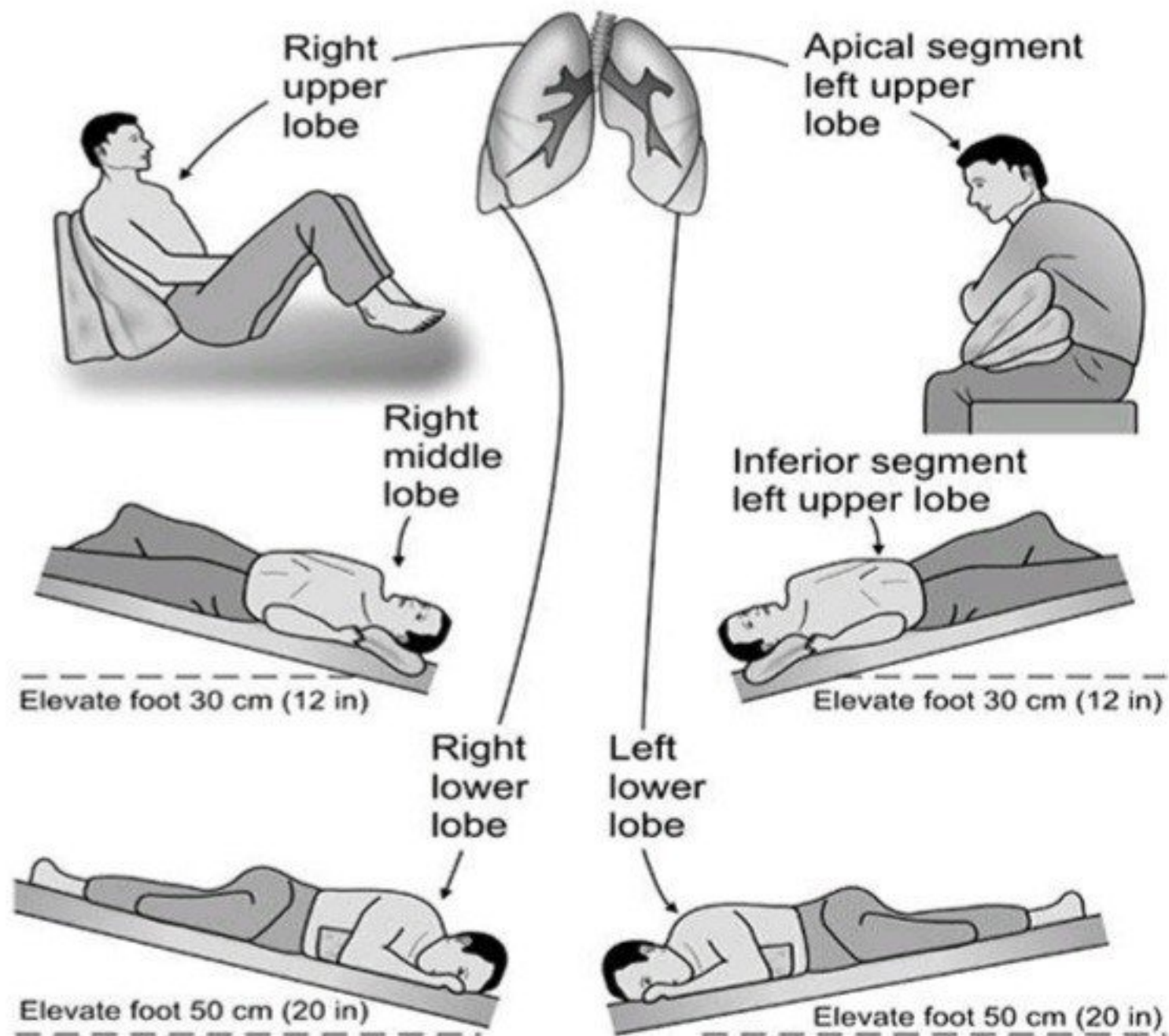
Vibration

ed after
on or in lieu of
exhalation
6-8 trials
with huffing or
g



Percussion and Vibration





UPPER LOBES

Apical Segments



Anterior Segments



Posterior Segments



MIDDLE LOBES

lingula



right

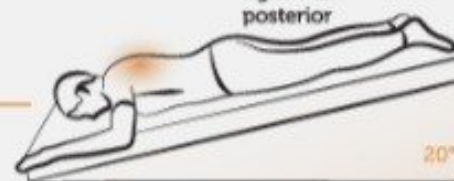


LOWER LOBES

Basal Segments



right + left posterior



Lateral Segments

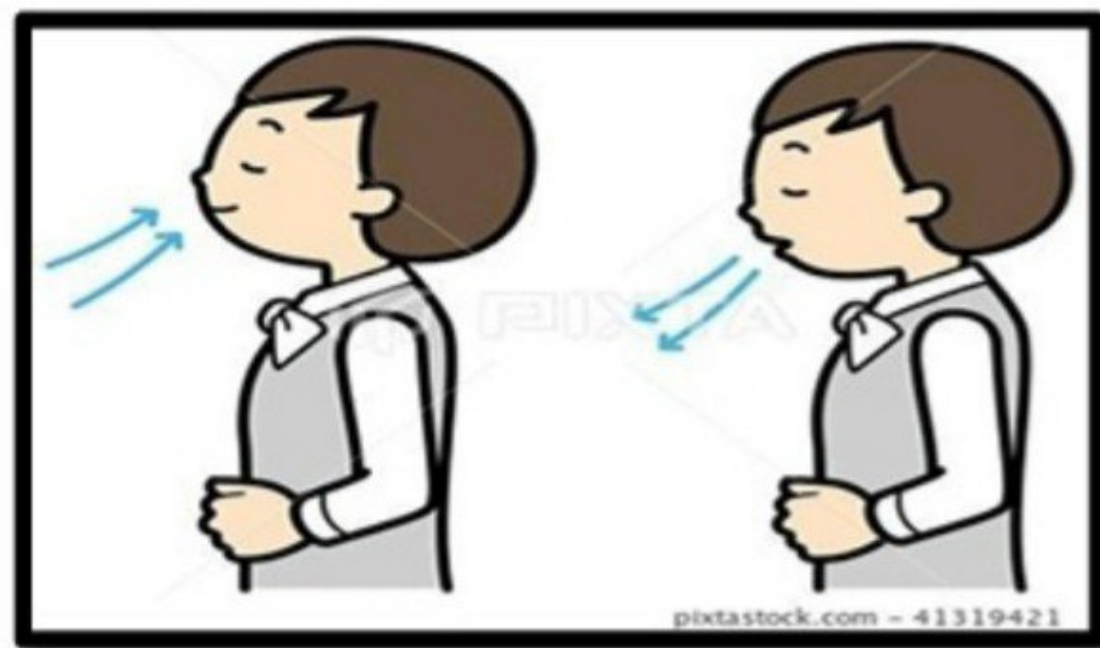
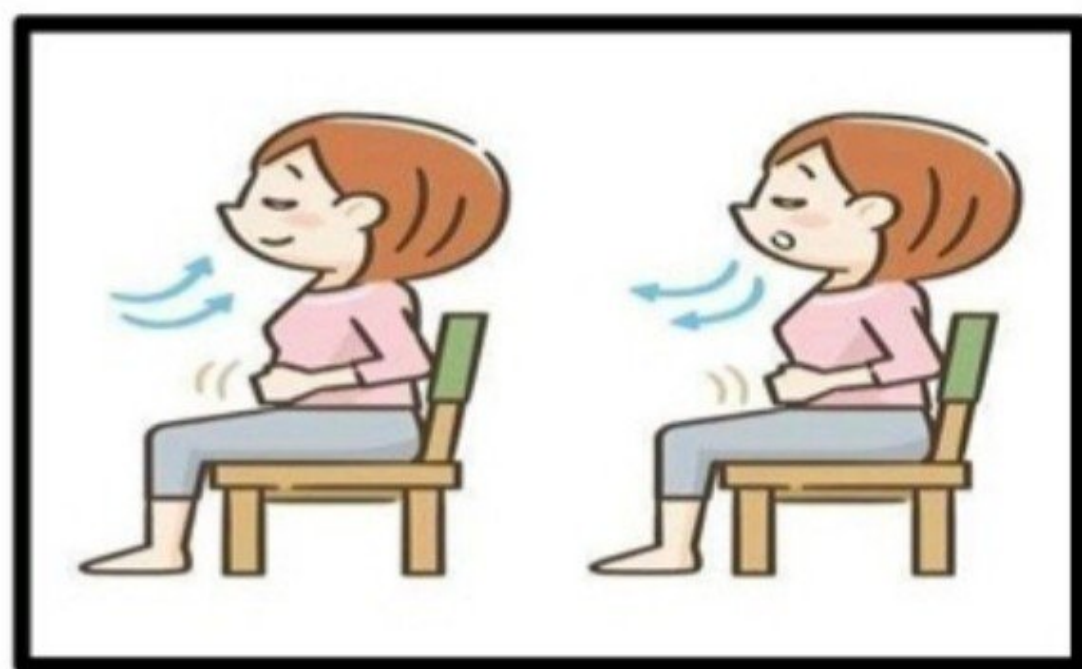


Superior Segments



XA





Deep breathing exercise

- This prevents the transmission of microorganisms.



Supplies/Equipment

- If either nostril is available, select the nostril closest to the patient's head.
- Assess foremost patent nostril.



Assess for the best nostril

- Patient's head position related to NG insertion to allow for passage of



Tu **Measure distance of** ing tap
 wa... soluble
 lubricating jel **the tube** y varies and
 Should be checked.



**Lubricate NG
 tube tip**

- This prevents displacement of the NG tube while checking placement.



Th
aci
ma
or small bowel content, and the tube should be removed.

Temporarily anchor



This
identi

Verify tube placement (PH Test)

on and
migration.